

The screening exists. Completion is what's missing.

A partnership brief on closing the gap between preventive care that is offered and preventive care that is actually completed — across Bangladesh's community-clinic screening system.

THE PITCH IN 30 SECONDS

PROPOSED · IN DEVELOPMENT

PROBLEM	Bangladesh's national screening program has reach and a registry — yet its own evaluation reports colposcopy follow-up compliance of only 52.7–57.9% . That is a completion-and-fidelity gap, not a science gap.
SOLUTION	A proposed disease-agnostic completion engine : it finds who is overdue, removes friction, and drives follow-through — with an explainable dashboard.
THE MODEL	Embed, don't build — the engine plugs into existing clinics and the national registry. In Bangladesh, friction reduction means the barriers that actually stop completion: transparent bundled pricing, literacy-tailored urgency communication, and one-visit / home-collection logistics.
EVIDENCE	Each mechanism carries quantified effect sizes from the CDC Community Guide; the program's own evaluation identifies the uptake and follow-up gaps the loop is designed to close.
THE ASK	A first private-network partner to run the loop commercially — then a donor-supported public-programme pilot in a few upazilas to prove it at national scale.

THE PROBLEM — ADOPTION, NOT SCIENCE

69%

of health spending is out-of-pocket (WHO, 2025) — one hospitalization can push a household into poverty.

<1% of GDP

public health spending (World Bank, 2021) — prevention is under-resourced.

52.7–57.9%

colposcopy follow-up compliance in the national screening program's own evaluation — nearly half of positives never complete follow-up.

WHICH ONE ARE YOU? — DIFFERENT PARTNERS, DIFFERENT VALUE, SAME ENGINE

PRIVATE HEALTHCARE

Raise entry-test → package conversion

You already run entry-test campaigns; the leak is conversion — price shock, urgency that never lands, second-visit logistics. The loop attacks all three. Coverage alone doesn't complete care; a designed loop does.

GOVERNMENT

Strengthen a program that exists

DGHS NCDC and the Community Clinic Health Support Trust — piloted in a few upazilas, closing gaps the program's own evaluation identifies.

RESEARCH & NGO

Reach, rigor, and credibility

icddr,b, BRAC, Marie Stopes — publishable, scalable evidence that opens donor funding.

SIX PILLARS, ONE COMPLETION LOOP

- Service redesign & friction reduction
 - Agentic & conversational outreach
- Interoperable data infrastructure
 - Algorithmic equity & governance
- Predictive AI, explainable by design
 - Implementation enablement

Identify → Reduce friction → Communicate (literacy-tailored) → Escalate (community-health-worker outreach) → Measure, incl. diagnostic follow-up after a positive result.

THE DEMONSTRATION

Embed the completion loop into a program that already exists.

Rather than build something new, the engine embeds into the existing network of roughly 14,000 community clinics and the national electronic registry — piloted in a few upazilas, aligned to the national NCD and screening strategy — to close the uptake and follow-up gaps the program's own evaluation identifies. Disease-agnostic by design: the same loop applies to cervical and breast screening, hypertension, and immunizations.